

An Autistic Child in Your Clinic

For family doctors, nurses & health professionals — caring for an autistic pre-school child (roughly ages 2–5).

The one idea

Autism, through **monotropism**, means a child's attention is pulled into a few deep, intense "tunnels" at a time. Inside a tunnel, focus is vivid and rich; outside it, things barely register — sometimes including pain, hunger, or someone speaking. Switching tunnels costs real energy (*autistic inertia*). That explains the strong interests, the sensory reactions, the need for routine, and meltdowns/shutdowns. The most useful thing you can do in a clinic is work **with** the tunnel, not against it.

What you may see — and what's really going on

You may see...	What's often happening
Won't make eye contact; looks away; seems to ignore you	Eye contact and split attention cost scarce resources; looking away helps the child regulate and listen
Deeply absorbed in one object; lines things up; plays "differently"	Monotropic play — valid and meaningful, not "wrong"; often the child's route to calm and learning
Sudden crying, yelling, going floppy or silent	An involuntary meltdown or shutdown from lights, noise, the wait or pain — <i>not</i> naughtiness
Doesn't say where it hurts; misses a bump or something hot	When absorbed, pain and internal signals may genuinely not register
Slow to respond; repeats your words back	Processing time + single-channel, literal communication

Try this

- **Read the family first.** Offer a short pre-visit form (the AASPIRE *Autism Healthcare Accommodations Tool*, autismandhealth.org) asking about sensory, communication and procedural needs — and read it before they arrive.
- **Book the quietest slot** (first or last), allow a trusted adult, and welcome their **sensory kit** (noise-cancelling headphones, sunglasses, a comfort or interest object).
- **Lower the load:** dim the lights, silence beeps where possible, skip strong perfumes.
- **Say what will happen, then do it** — short, literal sentences, one step at a time; show the equipment first.
- **Let the child look away, stim, and hold their interest object** during the exam. Join their interest to build rapport

before you examine.

Avoid this

- Forcing eye contact, holding the child down, or springing a procedure by surprise.
- Long verbal instructions or phrases like “be brave!” — say exactly what will happen, and mean it.
- Reading a calm, masked few minutes as “they’re fine” — a pre-schooler may pay for the visit with a shutdown afterwards.

When to get more support

Refer early for an autism assessment if you notice slowed / “sticky” attention, intense focused interests, sensory differences, or atypical play — early identification helps families adapt the environment, not the child. Watch for **autistic burnout** / “**regression**” under overwhelm: a loss of skills usually means *reduce demands and restore rest and interests*, not push harder. Always take **parental concern** seriously. Signpost the health visitor, paediatric services, and local autism-led parent support.

If the child is a girl

Girls are often better at early **camouflaging** and are missed more often. Take parental concern seriously even when the presentation looks “subtle” or “sociable” — masked effort in clinic is real effort.

About this guide

AI-assisted, derived from this project’s research drafts — the **Monotropism** brief and the **Lifespan Practitioner & Health-Care guide** (Parts 3.1 & 4). Uses identity-first language (“autistic child”).

Informational — not medical advice or a diagnostic tool. Monotropism is a powerful but still-developing theory; always fit it to the individual child.

Key sources. Murray, Lesser & Lawson (2005); Murray, F. (2018/2023); Dwyer (2025); AASPIRE Healthcare Toolkit / AHAT (autismandhealth.org); Raymaker et al. (2020, autistic burnout); Kelly Mahler (2024, interoception); Landry & Bryson (2004, sticky gaze). Full citations are in the source drafts.